

DISCHARGE SUMMARY: HEART FAILURE RECOVERY PLAN

Reason for admission

You were admitted with worsening shortness of breath and swelling in your legs caused by congestive heart failure (CHF). You were treated with intravenous diuresis and your oral medicines were adjusted. You are now stable, breathing comfortably at rest, and able to walk to the bathroom without distress.

Diagnoses

- Chronic congestive heart failure with reduced ejection fraction
- Hypertension

Medications at discharge

- Furosemide 40 mg by mouth once daily in the morning
- Lisinopril 10 mg by mouth once daily
- Metoprolol succinate 25 mg extended release by mouth once daily

Take your medications every day, even when you feel well. Do not stop the furosemide on your own, as it removes the extra fluid that strains your heart.

Daily self-care

Weigh yourself every morning after using the bathroom and before breakfast, using the same scale and similar clothing. Write the weight down. Limit salt to less than 2 grams per day and limit fluids to about 2 liters per day. Rest when you feel tired and elevate your legs when sitting.

Warning signs: when to get help

Call your care team the same day if you notice any of the following:

- Weight gain of more than 2 to 3 pounds in a single day, or more than 5 pounds in a week
- Increasing shortness of breath, or needing extra pillows to sleep
- New or worsening swelling in your legs or belly

Call 911 immediately if you have chest pain, severe trouble breathing, or fainting.

Follow-up care

Schedule a follow-up visit with your care team within the next two weeks. Bring your daily weight log and your medication list to the visit.